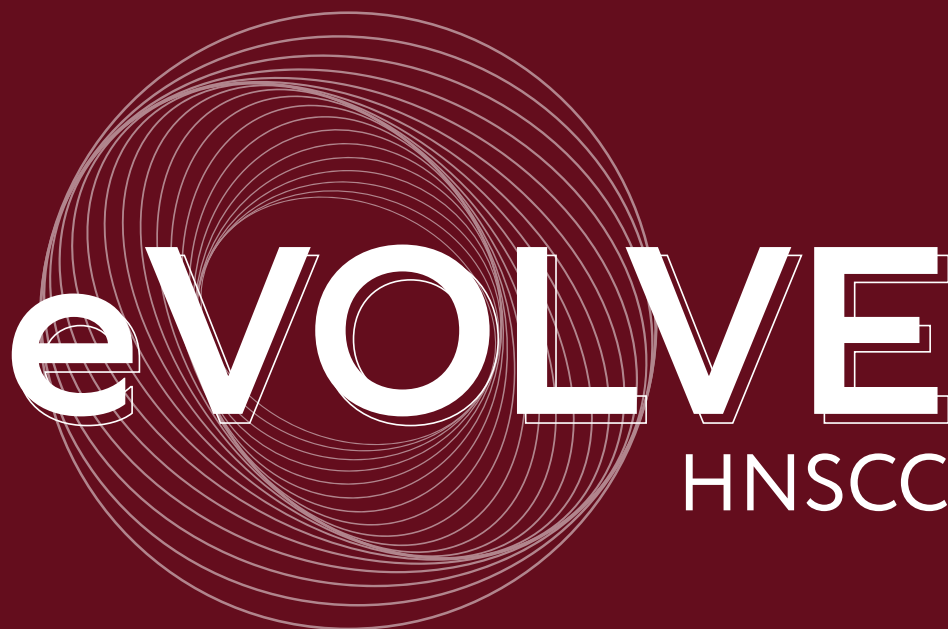
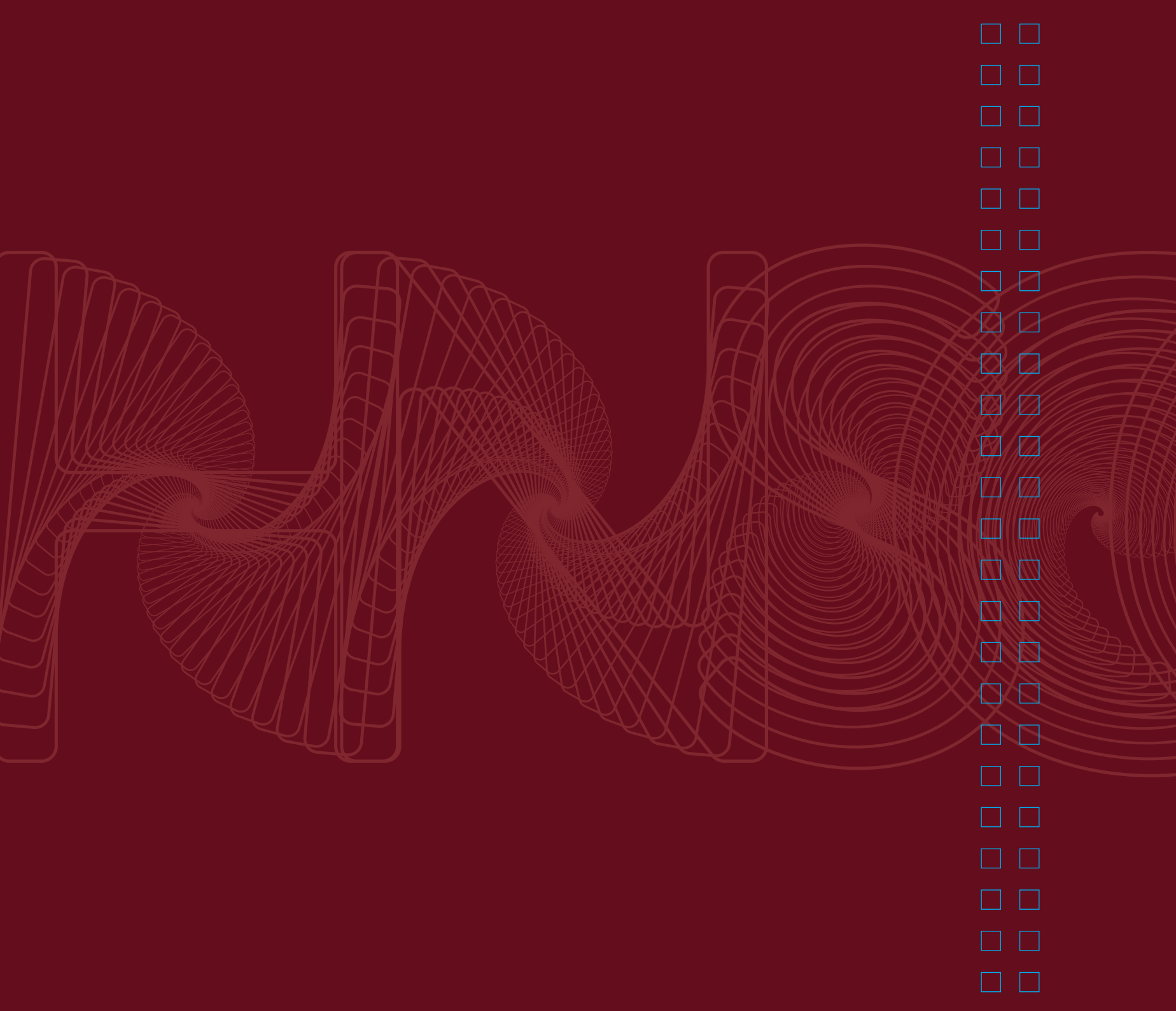




Your guide to the study

This guide has been developed to support you
during the eVOLVE-HNSCC clinical study
and explain what to expect.



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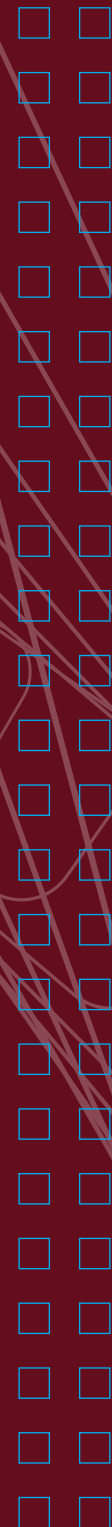
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About the study



1. Introduction

eOLVE-HNSCC is a clinical study involving people living with cancer that affects the head and neck which:

- Has spread to other nearby areas of the body or glands (called lymph nodes) in the neck
- Has not been removed by surgery



You may have heard your medical team call it **head and neck squamous cell carcinoma (HNSCC)**.

This guide is to help you understand more about:



The current treatments available for HNSCC and why you have been asked to take part in the study



What the study drug is and how it works



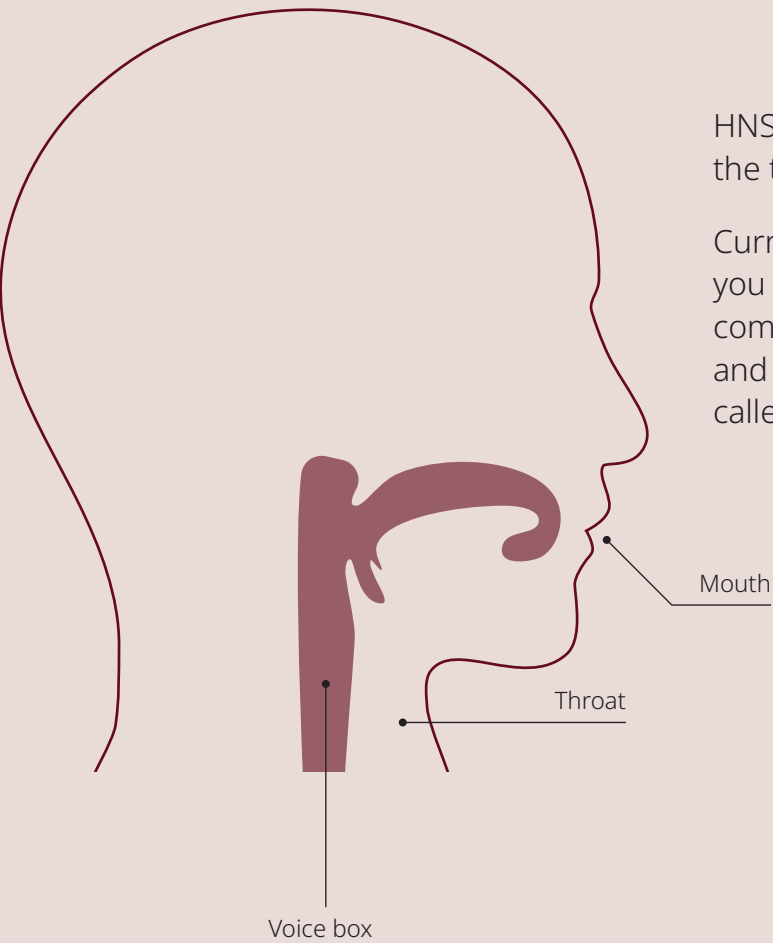
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2. Understanding HNSCC

When cells in the head and neck grow out of control, they can cause a type of cancer called **head and neck squamous cell carcinoma (HNSCC)**.



HNSCC can develop in or around the throat, voice box and mouth.

Currently, the type of cancer you have is treated with a combination of **chemotherapy** and **radiotherapy**. This is sometimes called **chemoradiotherapy**.



Chemotherapy: medicines designed to target cells that grow and divide quickly (like cancer cells)

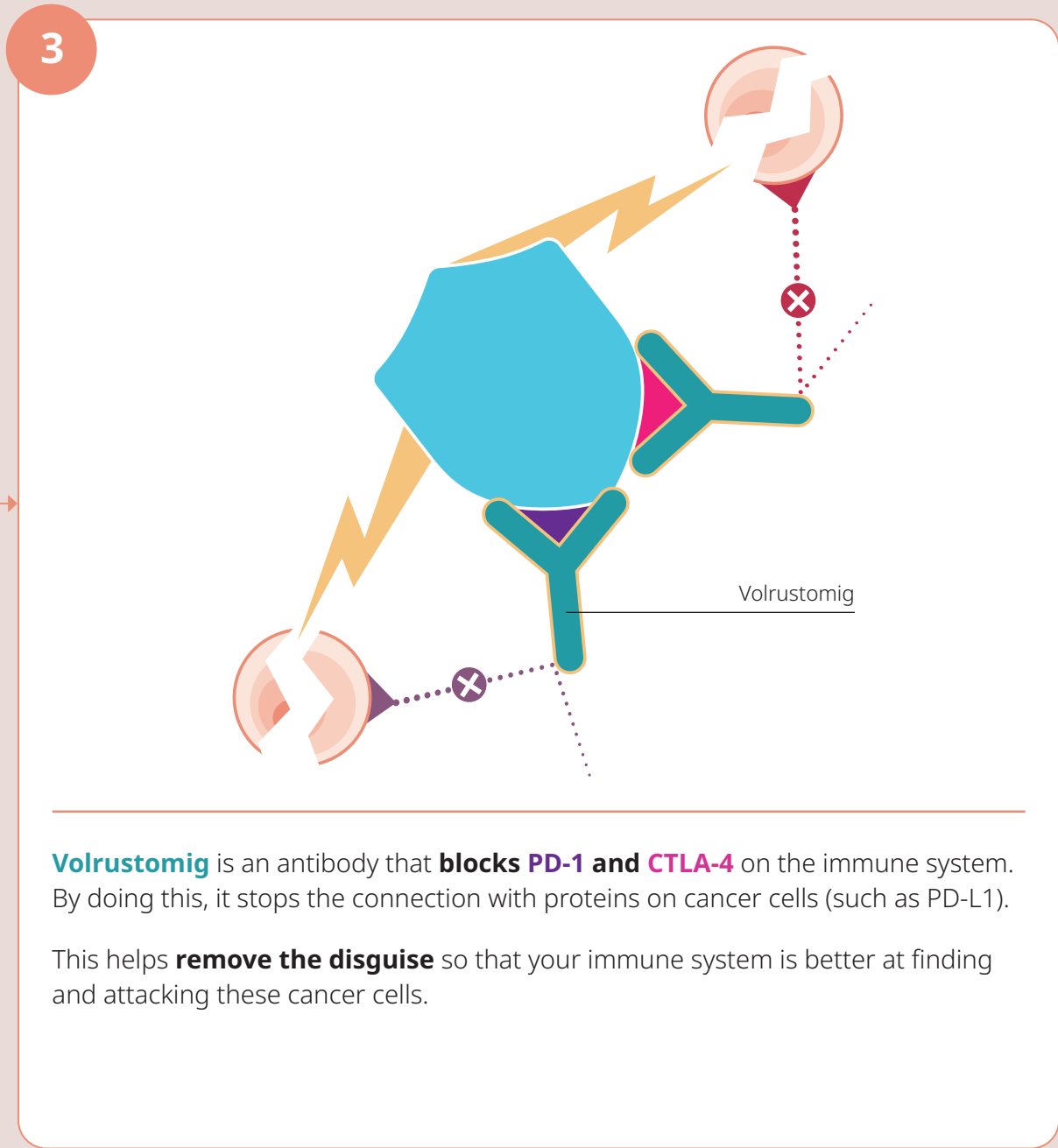
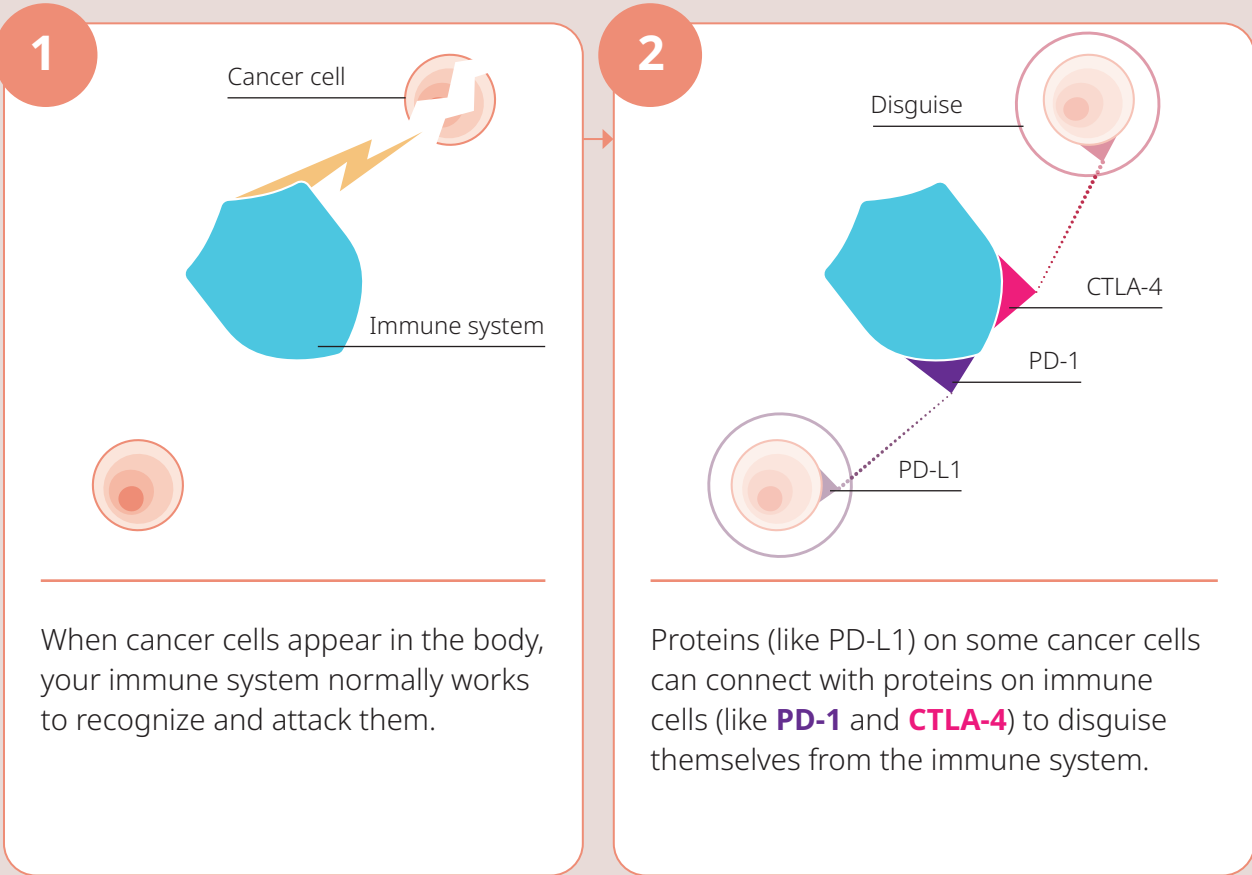


Radiotherapy: a type of treatment that uses intense beams of radiation to kill cancer cells

Before starting this study, you will **always** receive chemoradiotherapy.

3. What is the study treatment?

The eVOLVE-HNSCC study is investigating an experimental treatment called **volrustomig (vol-rust-oh-mig)** for people with your specific type of HNSCC.



4. What can I expect when I take part in this study?

The eVOLVE-HNSCC study is divided into 3 parts:



Screening

You will have some tests and assessments to see if the study is right for you



Treatment or observation

If the study is right for you, you will be placed into 1 of 2 groups



Follow-up

After the treatment or observation part has finished, you will be invited back to the study clinic so the team can continue to monitor your health and well-being

Screening

Screening is divided into 2 parts. **Both** will need to be completed.

Part 1

This may happen any time **before, during or after you receive chemoradiotherapy.**

- The study team will take a tumor sample* to check the amount of PD-L1 on your cancer cells
 - If you have a type of HNSCC that affects the throat (called oropharyngeal cancer), your tumor sample will also be tested for a virus called HPV (human papillomavirus)[†]
- These tests will help find out who is more likely to benefit from volrustomig

*The study team will try to use a recent sample taken less than 3 months before you started chemoradiotherapy. If this is not possible, you will be asked to have another tumor sample taken before you start chemoradiotherapy.

[†] More information on HPV can be found in the glossary at the back of this guide.

Part 2

After you have had chemoradiotherapy treatment, you will have the following tests and assessments as part of screening:



A physical examination (including your height and weight)



Vital signs



Heart tests (ECG/MUGA[‡])



Blood tests (including pregnancy test if applicable)



Urine tests



Scans of your tumor (CT, MRI or PET)[§]

[‡]ECG = electrocardiogram; MUGA = multigated acquisition scan.

^sCT = computerized tomography; MRI = magnetic resonance imaging; PET = positron emission tomography.

If the study is right for you, you will move into the treatment or observation part of the study.

More information about the tests and assessments can be found in your informed consent forms (ICFs).

You can speak to your study team at any time if you have any questions or concerns.

Study treatment or observation

The treatment or observation period will last up to **1 year**.
You will be assigned to 1 of 2 groups:



GROUP A:

Receive treatment with volrustomig.



GROUP B:

Continue being monitored by the study team after you have had chemoradiotherapy treatment during screening, but you will not receive any study treatment.

This means you will be monitored more closely and receive some extra tests and assessments than if you did not join the study. More information on what is involved for Group B can be found later in this guide.

The study group that you join will be chosen at random by a computer. This is to make sure that the study is fair and unbiased. Both you and your study team will know which group you join.

When you know which group you will join, you can use the tabs in this guide to help you easily go to the information that you might need.



What kind of tests and assessments will I have?

Tests and assessments are a valuable way of assessing your health and well-being and how the study treatments are working. Many of these will be similar to the ones you will have in the screening period.

Some of the tests and assessments that you will have during clinic may include:



A physical exam (including vital signs and weight)



Blood and urine tests (including pregnancy tests if applicable)



Heart tests (ECG*)



Scans (CT, MRI or PET†). These will take place:

- **Every 3 months** for **2 years**
- After this, they will be **every 4 months** for **another 2 years**
- **After 4 years**, you will have scans **every 6 months**

*ECG = electrocardiogram.
†CT = computerized tomography; MRI = magnetic resonance imaging;
PET = positron emission tomography.

What should I expect if I receive treatment with volrustomig?

Volrustomig is given as an injection into a vein over a period of time, known as an **intravenous infusion**.

Each infusion will be followed by a **3-week** period of rest. This is known as a **treatment cycle**.

You will have **18 cycles** in total, which are expected to last around **1 year**.



What side effects should I be aware of?

Like all medicines, there are some side effects that may be associated with taking volrustomig. They could happen during or after treatment.

A few side effects to be aware of are:

An overreactive immune system

This is where the immune system mistakes parts of the body for the cancer cells and tries to remove them. This can cause parts of the body, including the heart, liver, lungs, muscles and digestive system, to become swollen.

Your study doctor will discuss some of the symptoms of an overreactive immune system to look out for. They will also monitor your health and well-being while you are taking your study treatments.

Allergic reactions

Some people may have an allergic reaction to volrustomig. It could happen as soon as they have received treatment, or a few days after.

Allergic reactions have 1 or more of the following symptoms:



Fever



Chills



Rash



Low blood pressure



Swelling of the lips, mouth, throat or face



Difficulty breathing

[illegible]

If you experience any of the symptoms of an allergic reaction listed here, or think you are having an allergic reaction, **please go to hospital immediately.**

The team would like to emphasize that they are always here if you have any questions or concerns at any time.

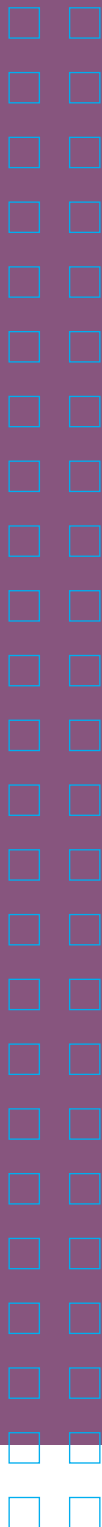
Please report any side effects or changes in your health to your healthcare team straight away.

Follow-up

When you stop receiving volrustomig, for whatever reason, you will be invited to come back to the study clinic for follow-up appointments.

You will have a visit **1 month** after you stop taking volrustomig. Then you will have another visit about **3 months** later.

At these appointments, your study doctor will check your health and well-being and carry out some tests that are similar to the ones you will receive in the screening and treatment periods. This will also be an opportunity to speak to your study doctor about any side effects you may be experiencing.



After this, you will be invited to have follow-up telephone appointments every 3 months to check your health and well-being.



Why is there an observation group and what will happen if I join it?

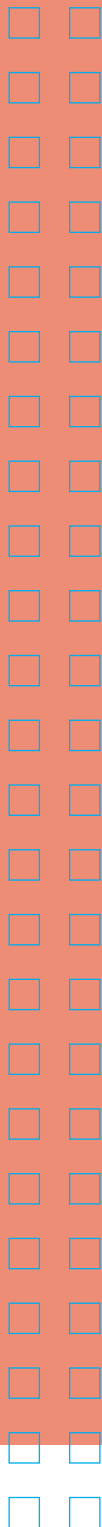
Sometimes, despite treatment, cancer can come back. For some types of cancer, patients can keep taking medicines to help lower the risk of this happening.

For your specific type of HNSCC, there currently aren't any medicines that can be taken to help lower the chance of the cancer coming back.

The next steps for people with your type of HNSCC are usually to have **regular check-ups and observations** after you have finished treatment with chemoradiotherapy.

eOLVE-HNSCC is looking into whether volrustomig can become an additional treatment, but it needs to be compared with the usual care for HNSCC in order for research scientists to understand how it works.

Since patients in this group will not be taking any ongoing medicines, it means that there will not be as many clinic visits to attend. This is to make sure your time in the study is as easy as possible.



If you join the observation group, you will receive the best supportive care that is available for your HNSCC, and you will have regular contact with the study team.

Observation period

Your study team will invite you to have appointments to discuss your health and well-being **every 6 weeks** for the **first year**.

Follow-up

You will be invited to have 2 appointments about 1 and 3 months after the observation period finishes. After this, you will be invited to appointments **every 3 months** to check on your health.

All appointments will be **virtual where possible**, online using a computer or telephone. Your appointments can also be arranged to be in person if you would prefer this option.

Remote appointments will have the same standard of privacy. In some cases, you may be asked to attend the clinic for a face-to-face appointment.

What tests and assessments will I have?

You will be asked to attend the study clinic for:



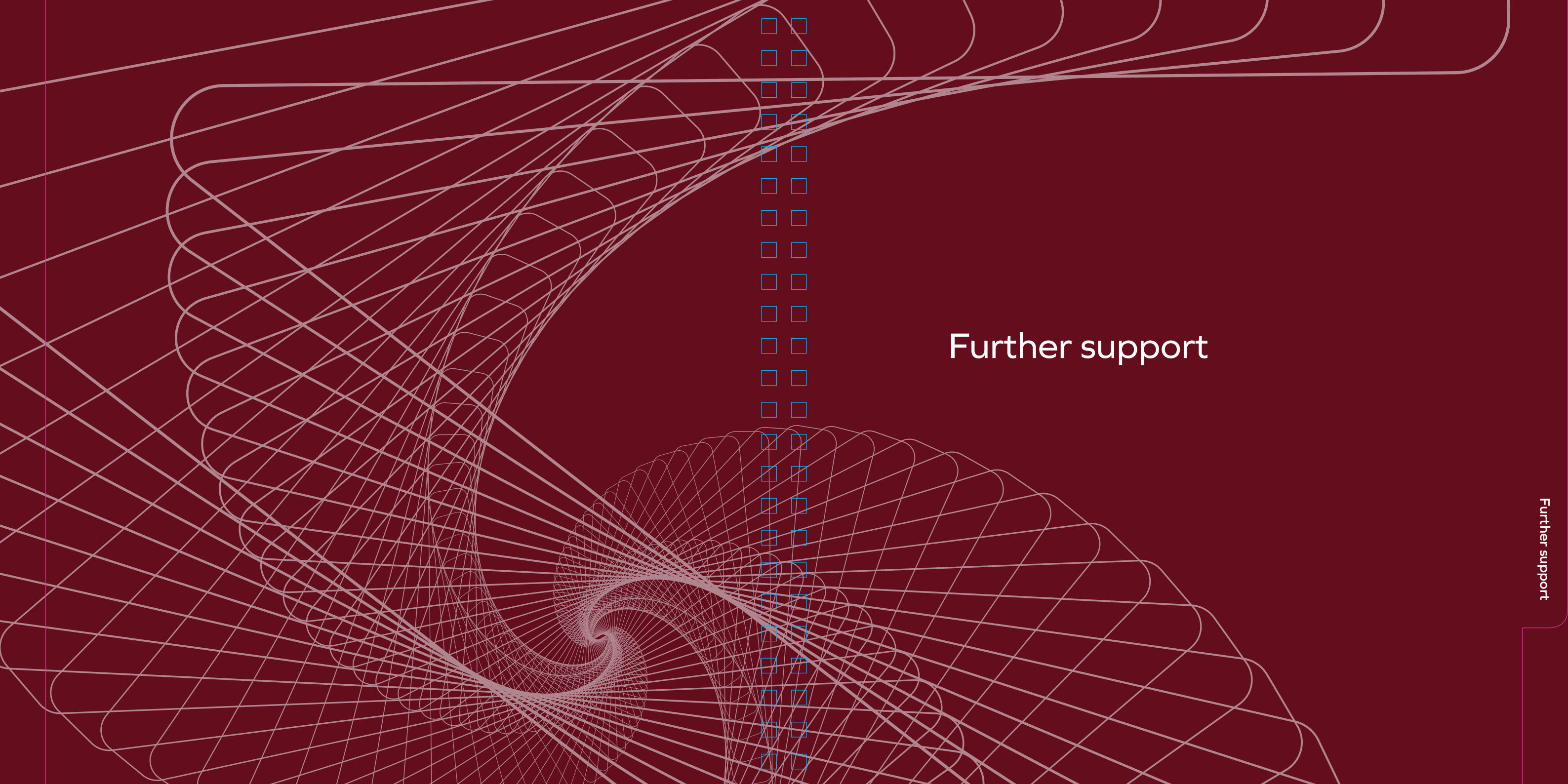
Blood and urine tests based on your health and well-being (if applicable)



Scans (CT, MRI or PET*). These will take place:

- **Every 3 months** for **2 years**
- After this, they will be **every 4 months** for **another 2 years**
- **After 4 years**, you will have scans **every 6 months**

*CT = computerized tomography; MRI = magnetic resonance imaging; PET = positron emission tomography.



Further support

5. Other things to know

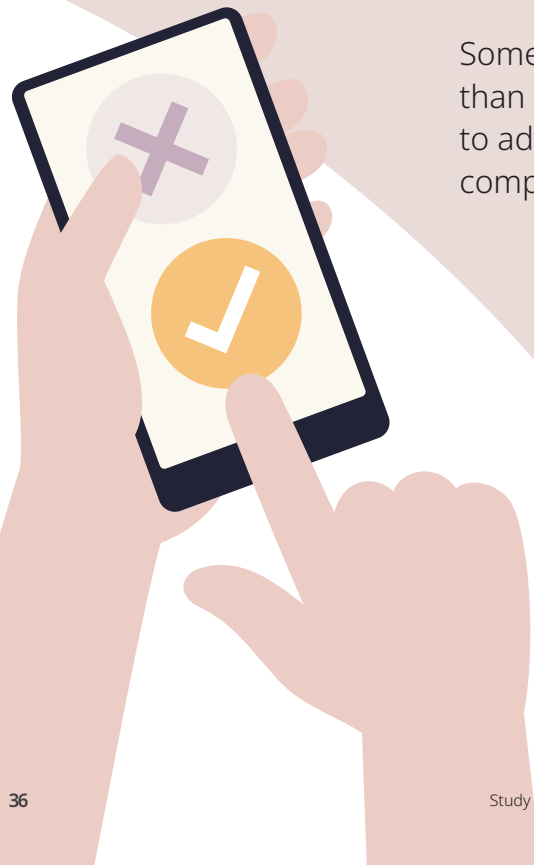
What tests will I have to do at home?

There are also some **questionnaires** about your symptoms and well-being that your study team will ask you to complete at home. These will need to be completed on a hand-held electronic device such as a **smartphone, tablet or computer**.

You will need to complete the questionnaires **every 1–3 weeks** depending on whether you are in Group A or B.

Some questionnaires will be less frequent than this. Your study team will be able to advise you on when you will need to complete each questionnaire.

Each questionnaire should take **2-15 minutes** to complete, but you can complete them at your own pace.



Optional tests and assessments

There may be a chance to have some more tests and assessments during treatment or observation, and follow-up. This is to help further our research. These are **optional**, and it is your choice if you would like to have them or not.

These include:



Tumor samples if your cancer gets worse

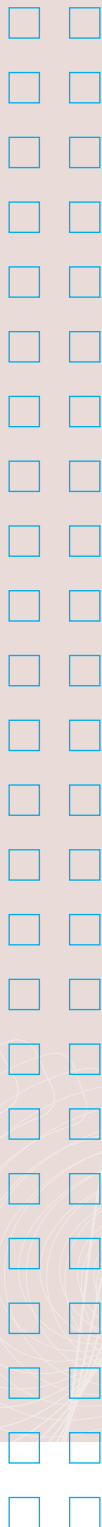
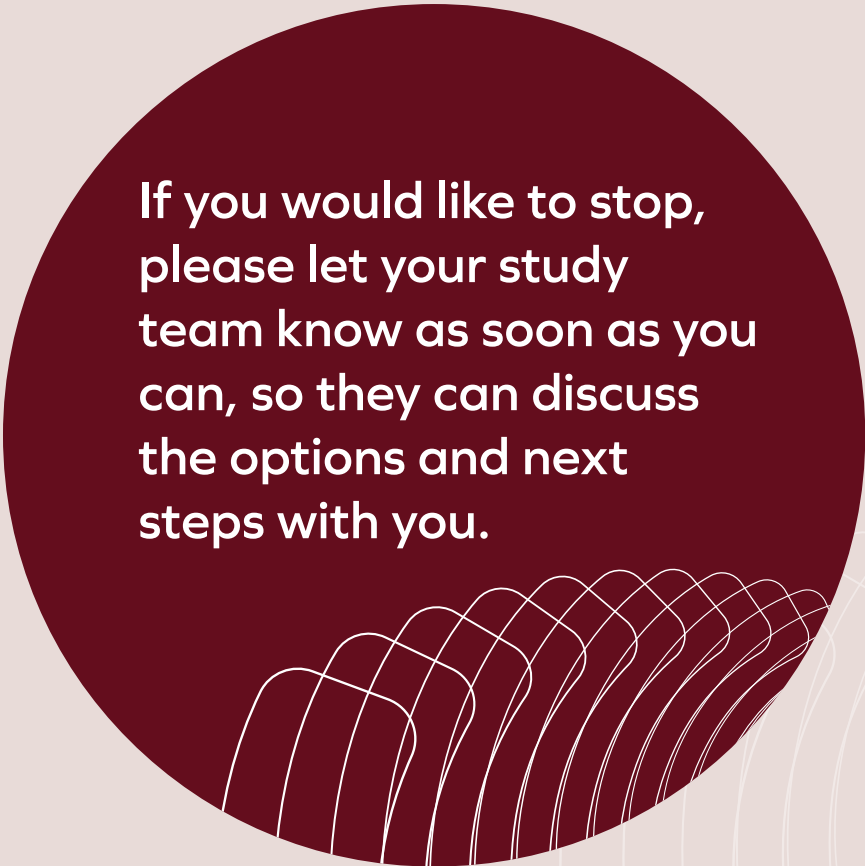


A single blood sample for genetic research

If you decide not to have them, you will still be able to join the study, and the quality of care that you receive will not be affected.

6. What happens if I change my mind?

Taking part in a clinical study is **always** your choice, and you can stop at any time, for any reason, and it will not affect the quality of care you receive.



Further support and information

If you have any questions or concerns, you can contact your study team at any time. You will also be given a Patient Participation/Information Card.

Study doctor

Name:

Phone:

Address:

Email:

Study coordinator

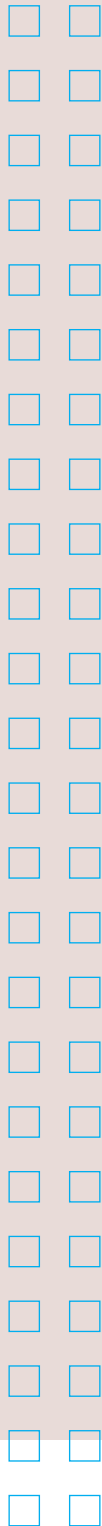
(e.g., nurse appointed to the study)

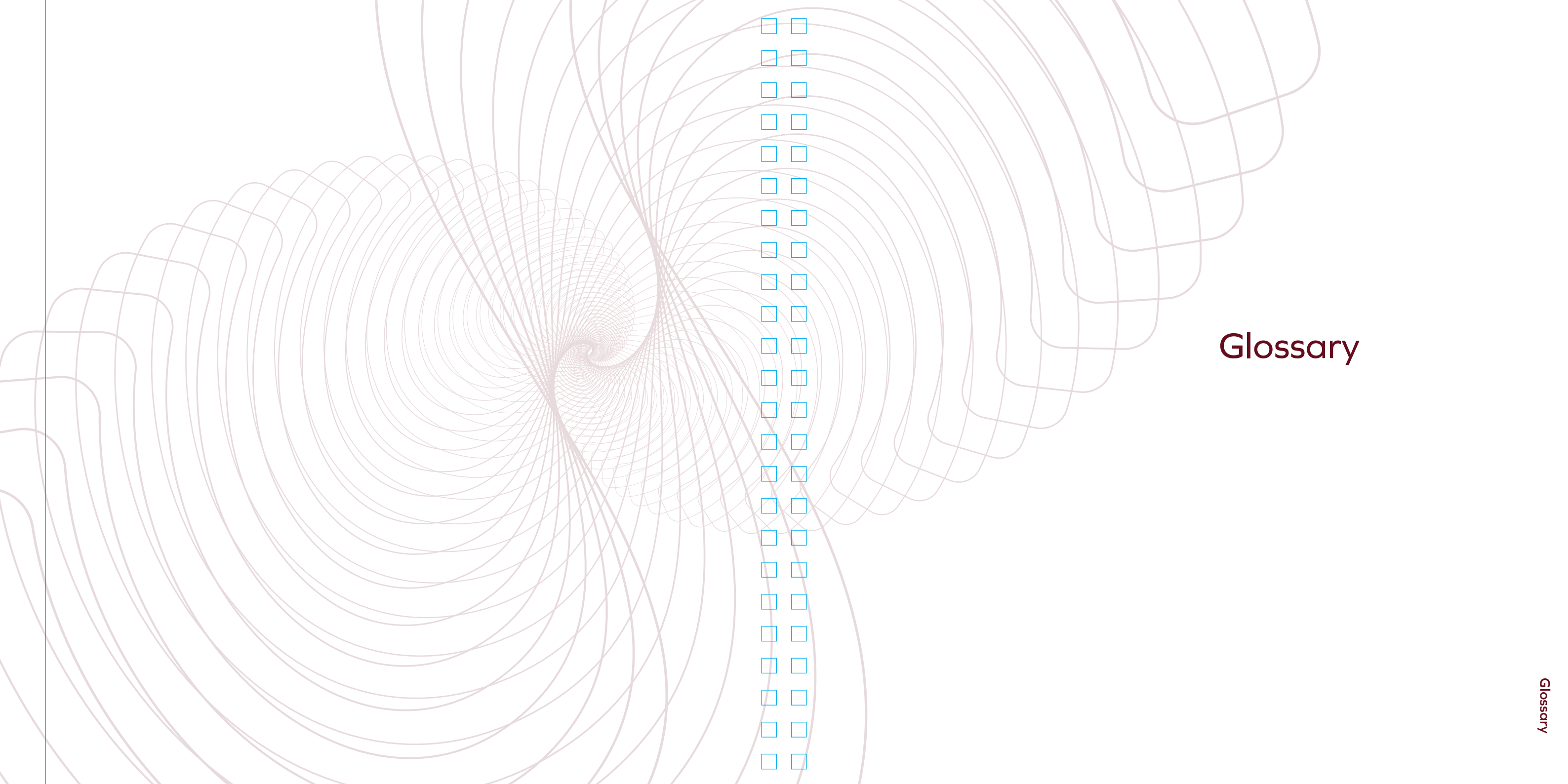
Name:

Phone:

Address:

Email:





Glossary

7. Glossary

Term	Description
Chemotherapy	A type of medicine that targets cancer cells and destroys them.
Chemoradiotherapy	A type of treatment that combines chemotherapy with radiotherapy.
Computerized tomography (CT)	A type of scan that uses a computer and X-rays to get detailed pictures of the inside of the body.
CTLA-4	A protein found on immune cells that helps cancer cells to disguise themselves from the immune system.
Electrocardiogram (ECG)	A test that checks the heart rhythm using an ultrasound machine.
Head and neck squamous cell carcinoma (HNSCC)	A type of cancer that is caused by cells in the head and neck growing out of control.

Human papillomavirus (HPV)	HPV is a group of common viruses that can be passed on through sexual activity, but usually don't have any symptoms. Most infections go away on their own, but sometimes if a person has a particular type of HPV, it can increase the risk of some types of cancer (including HNSCC).
Immune system	A system of organs and cells that helps protect the body from cancer and foreign substances such as bacteria, viruses and cancer.
Inflammation	A part of the body that becomes swollen. This can be caused by the immune system reacting to an injury or a medicine.
Lymph nodes	Small bean-shaped glands in the neck and head that are part of the immune system. Sometimes, lymph nodes can be affected by HNSCC.
Magnetic resonance imaging (MRI)	A type of scan that uses magnets to get detailed pictures of the inside of the body.

The study team would like to extend their deepest gratitude to you for taking part in eVOLVE-HNSCC. Studies such as this would not be possible without you.

The findings from this study will be used to help develop further treatments for people like yourself who are living with HNSCC.

